

CORRECTION NOTICE

Travel vaccines: Hepatitis A, Typhoid, Cholera · DOSING ERROR

The signed PGD behind this page is unaltered. This correction takes precedence over it.

Do not give Havrix 1440 as a 0.5 mL dose. The correct adult dose is 1.0 mL.

What this document says

·Havrix 1440 or Avaxim 160 - Inactivated Hepatitis A vaccine injection (1440 ELISA Units/0.5 mL or 160 antigen units/0.5 mL)·, dosed as ·0.5 mL single dose·.

Why that is wrong

Havrix 1440 is the ADULT presentation and contains 1440 ELISA units in 1.0 mL. 720 units in 0.5 mL is Havrix JUNIOR. The two products have been collapsed onto Avaxim's volume. Our own hepatitis A and B travel PGD states the correct figure:
·Havrix Monodose | 16 years and over | Hep A 1440 ELISA units | 1.0 mL·.

A pharmacist holding a 1.0 mL Havrix syringe and following this document expels half of it and gives half a dose. Avaxim 160 in 0.5 mL is correct and is unaffected.

Until a corrected version is signed

Give Havrix 1440 as the full 1.0 mL prefilled syringe. Give Avaxim 160 as 0.5 mL. Check the volume on the syringe against the product before administering. Anyone given 0.5 mL of Havrix 1440 has had a half dose and should be reviewed.

Also being corrected in the rebuild

This document contains no anaphylaxis provision of any kind: no requirement for adrenaline to be available, no training requirement and no observation period. Follow **your standard immunisation practice rather than this document on that point.** The records section does not require the batch number or the injection site. Record **both anyway; without a batch number a recall cannot be actioned.** All three arms are restricted to 18 years and over, which is narrower than the licences.

Issued 7 September 2026 · Get Real Health. Corrected version to follow after clinical review.

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Hepatitis A Vaccine (Havrix/Avaxim), Typhoid Vaccine (Typhim Vi), Cholera Vaccine (Dukoral) for the treatment of Travel Health

Summary of NICE / NICE CKS Guidelines for Travel Health

Pre-Travel Health Assessment

Pre-travel health assessment is essential for all travelers to identify and manage health risks associated with their destination, duration of travel, planned activities, and individual medical factors.

Risk Assessment

- **Destination and Duration:** Assess the epidemiological risks in the specific country or region, including current outbreaks and seasonal variations. Duration of stay and type of accommodation influence risk.
- **Activities:** Planned activities (trekking, water activities, visiting rural areas) may increase exposure to vaccine-preventable diseases.
- **Medical History:** Previous vaccinations, immunological status, chronic medical conditions, and medication use must be considered.

Assessment

- **Travel Itinerary Review:** Confirm specific countries/regions, duration, accommodation type, and planned activities.
- **Vaccination History:** Review existing immunity, previous travel vaccinations, and childhood immunisations.
- **Current Medications:** Review for interactions with vaccines and assess if immunosuppressed.
- **Medical Conditions:** Assess for contraindications, special precautions, or need for specialist advice.

Management

- **Vaccination Schedule:** Provide appropriate vaccinations according to travel risk, allowing adequate time between doses where required.
- **Preventive Measures:** Advise on food/water safety, insect bite avoidance, personal hygiene, and malaria prophylaxis if indicated.
- **Specialist Advice:** Consider referral for high-risk travelers or complex medical circumstances.

Red Flags and Referral Criteria

- **Immunocompromised Patients:** Seek specialist advice for HIV+, on immunosuppressants, or with primary immunodeficiency.
- **Pregnant Travelers:** Avoid live vaccines; discuss safe travel timing and other preventive measures.
- **Infants (<6 months):** Most vaccines contraindicated; refer for specialist advice.
- **Severe Allergies:** Consider alternative destinations or specialist pre-travel assessment.

Patient Group Direction
for the supply/administration of
Hepatitis A Vaccine (Havrix/Avaxim)
for the treatment of
Travel Health

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Protection against Hepatitis A for travelers to endemic areas
Inclusion criteria	Adults aged 18 years and over Traveling to areas with high or intermediate prevalence of Hepatitis A No previous complete Hepatitis A vaccination course No documented evidence of Hepatitis A immunity
Exclusion criteria	Known hypersensitivity to vaccine or any excipients Acute illness with fever (defer until recovered) Thrombocytopenia or coagulation disorders (risk of bleeding from intramuscular injection) Pregnant women (seek specialist advice)

Cautions	Caution in patients with thrombocytopenia (small needle preferred) Caution in patients taking anticoagulant therapy Caution in immunocompromised patients (may have reduced response; seek specialist advice) Give intramuscularly; do not administer intravenously or intradermally Vaccinate at least 2 weeks before departure if possible
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Havrix 1440 or Avaxim 160 - Inactivated Hepatitis A vaccine injection (1440 ELISA Units/0.5 mL or 160 antigen units/0.5 mL)
Legal category	POM
Storage	Store at 2-8°C. Do not freeze. Protect from light. Do not use if frozen.
Route/method of administration	Intramuscular injection in the deltoid region
Dose and frequency	Primary course: 0.5 mL single dose Booster dose: Required at 6-12 months after primary dose for long-term protection (10+ years)
Quantity to be administered and/or supplied	1 × 0.5 mL dose per visit
Maximum or minimum treatment period	Single dose for protection; booster dose required at 6-12 months
Adverse effects	Common (>1/100): Injection site reactions (pain, redness, swelling), headache, fatigue Uncommon (>1/1000): Myalgia, fever, nausea, abdominal pain Rare: Allergic reactions including anaphylaxis Very rare: Guillain-Barré syndrome, convulsions

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP - name of healthcare practitioner

	• name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with each vaccine. Ensure patient understands the importance of completing the full course if required and understands booster schedules.
Follow-up advice to be given to patient or carer	Complete the full vaccination course as recommended (particularly for Cholera vaccine which requires 2 doses). Attend for booster vaccinations as advised (Hepatitis A at 6-12 months; Typhoid every 3 years; Cholera every 2 years). Report any serious side effects to the GP or seek medical attention immediately. Maintain good food and water hygiene practices during travel (handwashing, avoid raw/undercooked foods, avoid tap water in endemic areas). Obtain comprehensive travel health insurance that covers medical evacuation if traveling to remote areas. Visit your GP or travel health clinic at least 4-6 weeks before departure for full pre-travel assessment. Report symptoms of Hepatitis A, Typhoid, or Cholera (fever, diarrhea, jaundice) to medical services immediately. If pregnant or planning pregnancy, discuss timing of vaccinations with your GP before travel.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>

- NICE CKS Guidance: Travel health
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

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Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Typhoid Vaccine (Typhim Vi)

for the treatment of

Travel Health

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Protection against Typhoid fever for travelers to endemic areas
Inclusion criteria	Adults aged 18 years and over Traveling to areas with high or intermediate prevalence of Typhoid High-risk groups: travelers to South Asia, Southeast Asia, Africa, or Central/South America Duration of travel: risk increases with longer stays in endemic areas
Exclusion criteria	Known hypersensitivity to vaccine or any excipients Acute illness with fever (defer until recovered) Thrombocytopenia or coagulation disorders Pregnancy (seek specialist advice)

Cautions	Caution in patients with thrombocytopenia Caution in immunocompromised patients (reduced response; seek specialist advice) Vaccinate at least 2 weeks before departure Not protective against paratyphoid A or B Effectiveness: 70-80% protective effect Revaccination recommended every 3 years for continued risk exposure
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Typhim Vi - Typhoid Vi polysaccharide vaccine injection (25 mcg/0.5 mL)
Legal category	POM
Storage	Store at 2-8°C. Do not freeze. Protect from light.
Route/method of administration	Intramuscular injection in the deltoid region
Dose and frequency	Primary course: 0.5 mL single dose Booster: Revaccination required every 3 years if continuing risk
Quantity to be administered and/or supplied	1 × 0.5 mL dose per visit
Maximum or minimum treatment period	Single dose for protection; revaccination every 3 years if required
Adverse effects	Common (>1/100): Injection site reactions (pain, induration, erythema), headache, myalgia Uncommon (>1/1000): Fever, fatigue, nausea, abdominal pain Rare: Lymphadenopathy, allergic reactions Very rare: Guillain-Barré syndrome, anaphylaxis

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP

	• name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with each vaccine. Ensure patient understands the importance of completing the full course if required and understands booster schedules.
Follow-up advice to be given to patient or carer	Complete the full vaccination course as recommended (particularly for Cholera vaccine which requires 2 doses). Attend for booster vaccinations as advised (Hepatitis A at 6-12 months; Typhoid every 3 years; Cholera every 2 years). Report any serious side effects to the GP or seek medical attention immediately. Maintain good food and water hygiene practices during travel (handwashing, avoid raw/undercooked foods, avoid tap water in endemic areas). Obtain comprehensive travel health insurance that covers medical evacuation if traveling to remote areas. Visit your GP or travel health clinic at least 4-6 weeks before departure for full pre-travel assessment. Report symptoms of Hepatitis A, Typhoid, or Cholera (fever, diarrhea, jaundice) to medical services immediately. If pregnant or planning pregnancy, discuss timing of vaccinations with your GP before travel.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
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Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional	Job title & name of	Signature	Date

group signatory	organisation		

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Cholera Vaccine (Dukoral)

for the treatment of

Travel Health

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Protection against Cholera for travelers to endemic areas with high risk of cholera
Inclusion criteria	Adults aged 18 years and over Traveling to areas with active cholera transmission or at high risk (particularly Africa, South Asia, parts of Latin America) Humanitarian workers, healthcare workers, or those with occupational exposure risk Travelers with planned extended stays in endemic areas with poor sanitation
Exclusion criteria	Known hypersensitivity to vaccine or any excipients (including formaldehyde, gentamicin)

	Acute illness with fever or gastrointestinal symptoms (defer until recovered) Pregnancy (seek specialist advice) Severe immunocompromise (though vaccine is inactivated)
Cautions	Caution with fever or acute gastrointestinal illness (defer until resolved) Take on empty stomach with water; do not mix with other liquids Antibiotics (if recently prescribed for enteric infection) may reduce vaccine effectiveness Caution with immunocompromised patients (may have reduced response) Vaccine does not prevent cholera due to other strains (limited cross-protection) Protection is partial (approximately 85-90% in first year, declining thereafter)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Dukoral - Oral inactivated cholera vaccine containing recombinant B subunit (rCTB) and inactivated whole cells
Legal category	POM
Storage	Store at 2-8°C. Do not freeze. Protect from light. Keep buffer sachet sealed until use.
Route/method of administration	Oral administration. Mix vaccine suspension with buffer solution; take on empty stomach.
Dose and frequency	Primary course: 2 doses, 1-6 weeks apart Booster: Single dose every 2 years if continuing risk in endemic area
Quantity to be administered and/or supplied	2 × doses for primary course (each course: 1 × 1.5 mL dose mixed with buffer)
Maximum or minimum treatment period	Primary course requires 2 doses over 1-6 weeks; booster every 2 years if required
Adverse effects	Common (>1/100): Abdominal pain, diarrhea, nausea, vomiting, loss of appetite Uncommon (>1/1000): Headache, fatigue, dizziness, fever, myalgia Rare: Vomiting, constipation, allergic reactions

	Very rare: Anaphylaxis, severe allergic reactions
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

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Follow-up advice to be given to patient or carer	Complete the full vaccination course as recommended (particularly for Cholera vaccine which requires 2 doses). Attend for booster vaccinations as advised (Hepatitis A at 6-12 months; Typhoid every 3 years; Cholera every 2 years). Report any serious side effects to the GP or seek medical attention immediately. Maintain good food and water hygiene practices during travel (handwashing, avoid raw/undercooked foods, avoid tap water in endemic areas). Obtain comprehensive travel health insurance that covers medical evacuation if traveling to remote areas. Visit your GP or travel health clinic at least 4-6 weeks before departure for full pre-travel assessment. Report symptoms of Hepatitis A, Typhoid, or Cholera (fever, diarrhea, jaundice) to medical services immediately.

	If pregnant or planning pregnancy, discuss timing of vaccinations with your GP before travel.
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